



P 52 · 40 PAGES, PRINTABLE + FILLABLE

## *The Birth Anxiety Workbook*

*A forty-page workbook for the fear of giving birth. Validates the fear, names the specific sub-fears, and offers tools that are not platitudes. Two-page workbooks for the six biggest fears, pain, loss of control, mom-complications, baby-complications, interventions, the unknown. Scripts for telling the team you are scared. The night-before-labor page. Routing to perinatal mental-health care when the anxiety is bigger than the workbook can hold. Crisis lines.*

---

## *Soothemade* Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

---

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

---

# *A note before you start*

---

**Y**ou are afraid of giving birth.

You have been told to “trust your body.” You have been told “women have done this for thousands of years.” You have been told to “just relax.” None of those have helped, because the fear is not irrational. Birth is a real event with real intensity and real risks. The fear is your nervous system paying attention to what is coming.

This workbook treats the fear seriously. It names what you are actually afraid of, in specific. It gives tools that are not platitudes. It gives you scripts for the conversations you need to have, with your provider, with your partner, with yourself.

It does not promise that the birth will go your way. No book can. What it does is give the fear somewhere to go besides the pit of your stomach at 3am.

Soothemade

*This is a workbook, not therapy. If your birth anxiety is interfering with eating, sleeping, daily function, or your bond with the pregnancy, please see a perinatal mental-health therapist. They specialize in exactly this. Crisis lines are at the back of the workbook.*

# *How to use this workbook*

---

- Read the “Birth IS scary” page first.
- Do the fear inventory. Find out what you are actually afraid of.
- For each major fear you have, work through the two-page section.
- Bring the workbook to your provider, your doula if you have one, your therapist if you have one.
- Some of the work needs another person. The workbook tells you when.
- Re-do the inventory in a few weeks. The fears can shift.

A small principle: anxiety hates specificity. “I am afraid of birth” is a fog. “I am afraid of being unable to communicate during transition because I have read that some people can’t” is a specific worry that can be talked about. Specificity lets the fear be addressed.

# *Birth IS scary*

---

**T**his page is permission.

## THE THINGS YOU HAVE BEEN TOLD

"Don't worry about it."

"It'll be fine."

"Trust your body."

"Women have been doing this for thousands of years."

"Your body knows what to do."

"It's a beautiful experience."

"Don't psych yourself out."

## NONE OF THESE ACKNOWLEDGE THE TRUTH

The truth is:

Birth is an intense physical event. It is one of the most physically demanding things a human body does. Many people describe the pain as the most intense they have ever experienced. Some experience trauma during it. Some experience complications. Some experience joy. Sometimes all of those in the same labor.

Modern obstetric care has reduced the risks dramatically over the last century. It has not eliminated them. The risks for you depend on your specific situation, your specific health history, and which team you give birth with.

You are allowed to be scared. The fear is data. It is your nervous system telling you that something big is coming. You can hold the fear and prepare for the birth at the same time.

The opposite of the "just relax" advice is not "be paralyzed by fear." It is "be informed. Be clear about what you want. Be prepared to ask for what you need. Have people who will advocate for you."

That is the work this workbook helps with.

## *The fear inventory*

---

**N**ame the fears. Specifically. The vague “I am afraid of birth” is harder to work with than the specific list.

WHICH OF THESE ARE TRUE FOR YOU (check all that apply)

#### PAIN

- ☐ I am afraid the pain will be unbearable.
- ☐ I am afraid I will be unable to cope.
- ☐ I am afraid of needles and medications used for pain relief.
- ☐ I am afraid I will not be able to get pain relief when I want it.
- ☐ I am afraid I will need stronger pain relief than I planned for.
- ☐ I am afraid the pain will harm me long-term.

#### LOSS OF CONTROL

- ☐ I am afraid of being unable to make decisions during labor.
- ☐ I am afraid of being pressured into decisions.
- ☐ I am afraid of being unable to communicate.
- ☐ I am afraid of losing dignity or modesty.
- ☐ I am afraid of being naked in front of strangers.
- ☐ I am afraid of needing a c-section when I planned a vaginal birth.

#### COMPLICATIONS FOR ME

- ☐ I am afraid of dying.
- ☐ I am afraid of having a serious complication.
- ☐ I am afraid of hemorrhage.
- ☐ I am afraid of preeclampsia or eclampsia.
- ☐ I am afraid of a long recovery.
- ☐ I am afraid of permanent injury (incontinence, prolapse, scars).

#### COMPLICATIONS FOR THE BABY

- ☐ I am afraid of the baby being hurt during labor.
- ☐ I am afraid of the baby being stillborn.
- ☐ I am afraid of the baby needing NICU.
- ☐ I am afraid of a birth defect being discovered.
- ☐ I am afraid of cord issues.

#### INTERVENTIONS

- ☐ I am afraid of an episiotomy.
- ☐ I am afraid of tearing.
- ☐ I am afraid of forceps or vacuum-assisted birth.
- ☐ I am afraid of an unwanted c-section.
- ☐ I am afraid of an induction.
- ☐ I am afraid of being pushed into Pitocin or other interventions.

### PEOPLE IN THE ROOM

- ☐ I am afraid of my provider not listening to me.
- ☐ I am afraid of my partner's behavior during labor.
- ☐ I am afraid of strangers being in the room.
- ☐ I am afraid of a doctor I do not know being on call.
- ☐ I am afraid of being treated dismissively because of who I am.

### POSTPARTUM

- ☐ I am afraid of the recovery.
- ☐ I am afraid of breastfeeding.
- ☐ I am afraid of postpartum depression.
- ☐ I am afraid I will not bond with the baby.
- ☐ I am afraid of going home with the baby.

### THE UNKNOWN

- ☐ I am afraid because I have never done this before.
- ☐ I am afraid because the last birth was traumatic.
- ☐ I am afraid because I do not know what to expect.
- ☐ I am afraid of something I cannot quite name.

### OTHER

---

---

### MY TOP THREE FEARS, SPECIFICALLY

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_

For each of these top three, work through the corresponding 2-page section. Then come back to this list and see what has shifted.



## *Fear of pain, page 1*

---

### WHAT, SPECIFICALLY, AM I AFRAID OF ABOUT THE PAIN

- ☐ I am afraid it will be more than I can handle.
- ☐ I am afraid I will not be able to communicate through it.
- ☐ I am afraid I will lose myself in it.
- ☐ I am afraid of having to ask for relief.
- ☐ I am afraid relief will not work.
- ☐ I am afraid of the medications used.
- ☐ Other: \_\_\_\_\_

### THE FACTS I KNOW

- Modern hospitals have many options for managing pain in labor. The full menu includes movement, position changes, water, breathing, counter-pressure, IV medications, epidural, and more.
- The right option for you is the one you and the team choose together. You can change your mind during labor.
- You can ask for pain relief at any point. You do not need to "earn" it by suffering first.
- Many people who planned an unmedicated birth choose pain relief during labor. Many who planned an epidural end up not getting one. Either way is valid.

### WHAT MY PROVIDER HAS TOLD ME ABOUT THE PAIN-RELIEF OPTIONS AT MY HOSPITAL

---

---

### WHO I WANT TO BE THE ONE WHO ASKS FOR PAIN RELIEF FOR ME, IF I AM UNABLE TO

---

### THE CODE WORD OR PHRASE I AM RESERVING FOR "I AM SERIOUSLY READY FOR RELIEF" (so people in the room know it's real)

---

## *Fear of pain, page 2*

---

THE TOOLS I WILL HAVE WITH ME (in addition to whatever pharmacological options I choose)

- ☐ A doula trained in labor support
- ☐ A partner who knows my preferences
- ☐ A playlist (or specific music I want)
- ☐ Specific lighting (dim, candles if allowed, no fluorescents)
- ☐ A focal point or object
- ☐ A scent (essential oil, if allowed, that is mine)
- ☐ A blanket from home
- ☐ A picture of someone or somewhere that grounds me
- ☐ Practiced breathing technique
- ☐ Position changes the team supports

#### PRACTICE BEFOREHAND

The "tools" only work if you have practiced them. The breath you did once in a yoga class is not the breath that will help you in labor. Either practice a specific technique you have learned, or release the idea that breathing alone will do the work.

What I have practiced:

---

#### THE THING I WANT MY PARTNER TO REMEMBER ABOUT PAIN

"You are allowed to ask for relief. You do not have to be brave. You do not have to earn anything. The baby will arrive whether or not you have an epidural. The provider will give you what you ask for."

When my partner sees me at the threshold, what I want them to do:

---

---

## *Fear of loss of control, page 1*

---

### WHAT LOSS OF CONTROL LOOKS LIKE, FOR ME

- ☐ Being unable to think clearly.
- ☐ Being unable to speak for myself.
- ☐ Being pressured into a decision quickly.
- ☐ Strangers entering the room without warning.
- ☐ Being told to do something I do not want to do.
- ☐ Being too tired to advocate for myself.
- ☐ Being naked or exposed.
- ☐ Other: \_\_\_\_\_

### THINGS I CAN CONTROL IN ADVANCE

- ☐ Who is in the room (partner, doula, family, support person)
- ☐ Who is NOT in the room (with the team's understanding)
- ☐ A birth plan that names my preferences in writing
- ☐ A doula whose job is partly to advocate
- ☐ Practiced statements I can use even when tired
- ☐ A code phrase between me and my partner for "stop and check in with me, do not let them rush me"

### THE BIRTH PLAN I HAVE OR AM WORKING ON

See P14 Birth Plan Pack for the full template.

The most important preferences I want to bring:

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_
4. \_\_\_\_\_
5. \_\_\_\_\_

### WHO IS THE PERSON IN THE ROOM WHO ADVOCATES FOR ME WHEN I AM TOO TIRED

\_\_\_\_\_

## *Fear of loss of control, page 2*

---

## A REMINDER ABOUT WHAT YOU CAN ASK FOR

You can ask the team to:

- Pause and explain anything they propose
- Give you a minute before a decision
- Bring in your partner before a non-emergency decision
- Tell you what each thing they are doing is for
- Lower their voices
- Knock before entering (this can be requested politely)
- Limit the number of people in the room
- Use specific words you prefer (and avoid words you do not)

You can DECLINE:

- Routine interventions you have explicitly opted out of in your birth plan, in the absence of medical emergency
- Specific staff being in the room (within reason)
- Procedures that are non-emergency and you have not consented to

You can REQUEST:

- A second opinion before a non-emergency decision
- Reading time before signing a non-emergency consent
- A specific provider when possible

## WHAT TO SAY IF YOU FEEL PRESSURED

"I'd like a moment to think about this."

"Can you explain what happens if we wait."

"I want my partner to be in this conversation."

"I am not consenting to that right now."

"I want to talk with the [doula / patient advocate / charge nurse]."

## A NOTE ON EMERGENCIES

In a true emergency, you may not have time for all of the above.

That is real. What the team is doing in those moments is often the right thing. The fear here is not about emergencies. It is about the non-emergency moments where you should have had time and were not given it. Those moments are the ones to be ready for.



## *Fear of complications for me, page 1*

---

THE SPECIFIC COMPLICATION I AM AFRAID OF

---

WHY THIS ONE, SPECIFICALLY (a story I read, a friend's experience, my own history)

---

---

WHAT MY PROVIDER HAS SAID ABOUT MY OWN RISK

---

---

WHAT THE TEAM WATCHES FOR (in plain language, in their words)

---

---

WHAT THE EARLY-WARNING SIGNS ARE (so I can speak up)

---

---

WHEN TO CALL THE PROVIDER BEFORE LABOR EVEN STARTS

- ☐ Sudden severe headache
- ☐ Vision changes
- ☐ Sudden swelling, especially face and hands
- ☐ Severe upper-right belly pain
- ☐ Bleeding
- ☐ Decreased baby movement
- ☐ Specific signs my provider has named:

---

## *Fear of complications for me, page 2*

---

## A REMINDER

Modern obstetric care has greatly reduced maternal mortality from where it was a century ago. The reduction has been uneven across demographics and locations, and your specific risk profile matters.

If you are at higher risk (your provider knows your specifics, but common factors include certain chronic conditions, prior pregnancy complications, certain demographic factors), your team will be more attentive, and the right move is to have a higher-care setting, more frequent monitoring, and clear escalation pathways.

## WHAT I WANT TO ASK MY PROVIDER ABOUT MY SPECIFIC RISK

---

---

---

## THE ESCALATION PATH I WANT TO UNDERSTAND BEFORE LABOR

- If X happens, what's the next step?
- Who do I call after hours?
- Where is the nearest higher-level care if needed?
- What does the team know about my history?

## WHO ELSE NEEDS TO KNOW MY MEDICAL HISTORY IN THE ROOM

- ☐ Partner (knows what to say if I cannot)
- ☐ Doula
- ☐ A document in my hospital bag with key history
- ☐ Emergency contact who can speak on my behalf

## A WORD ABOUT THE STATISTICS

Statistics are about populations, not about you specifically.  
"The risk is 1 in X" is not the same as your individual risk.  
Your provider's read of your situation is what matters.  
Statistics that scare you in the wrong direction are also worth talking through with your provider. Sometimes the internet's number is wrong for you.

## *Fear of complications for the baby, page 1*

---

## THE SPECIFIC FEAR

---

---

## WHEN THIS FEAR IS LOUDEST

- ☐ At night when the baby has been quiet
- ☐ After a scary story online
- ☐ Around the gestational age of a prior loss
- ☐ Approaching the appointment
- ☐ During labor itself
- ☐ Other: \_\_\_\_\_

## WHAT TO DO WHEN THE FEAR IS LOUDEST

1. Note what you have noticed about the baby (or not noticed).  
Decreased movement? Cramping? Bleeding? Anything physical?
2. If anything physical is concerning, call the provider's after-hours line. This is what it exists for.
3. If nothing physical is happening, the fear is the thing.  
The fear can be sat with. The fear is not a prediction.
4. Do not search online. Search makes it worse.
5. Call a person who knows you. Partner, friend, doula.

## KICK COUNT ROUTINE (if recommended for you)

When my provider has me do kick counts:

---

What "decreased movement" means to me, in my own words:

---

The phone number I will call if I am unsure:

---

## WHAT I AM PREPARING IF SOMETHING DOES GO WRONG

This is not pessimism. This is planning. Knowing P40 (Rainbow Baby) and P20 (Pregnancy Loss) exist as Soothemade products is itself a form of permission to feel the fear without it owning you.

Who I would call first: \_\_\_\_\_

Who I would tell second: \_\_\_\_\_

What I would want, right then: \_\_\_\_\_

## *Fear of complications for the baby, page 2*

---



## THE TRUTH ABOUT THE FEAR

Most pregnancies result in a healthy baby. Most concerning moments during pregnancy turn out to be okay. Most monitoring findings that prompt closer watch resolve into a normal birth.

Some don't. That is the part the fear is responding to. The fear is not stupid. It is the cost of loving someone you have not met yet.

What helps:

- Trust the provider's monitoring. They are watching for the things you are afraid of. The technology + the human team are doing their job.
- Know the signs that warrant a call. Decreased movement after week 28 is the big one. Bleeding. Severe abdominal pain. Anything your provider has named for your specific situation.
- Have a routine for when the fear hits at 3am: a glass of water, a hand on the bump, paying attention to movement, a slow exhale. If no movement, call the line.
- Limit the inputs. Stop reading stories about loss in the middle of the night. The brain stores them and brings them back later.

## WHAT I WILL DO THE NEXT TIME THIS FEAR HITS

---

---

A REMINDER: THE FEAR DOES NOT MAKE THE FEARED THING MORE LIKELY. THE FEAR IS A FEELING. THE FEELING IS REAL. THE FEELING IS NOT A PREDICTION.

*Fear of interventions you might not want,*  
*page 1*

---

## THE INTERVENTIONS I AM MOST AFRAID OF

- ☐ Induction when I planned to go into labor naturally
- ☐ Pitocin / synthetic labor augmentation
- ☐ Breaking my water artificially
- ☐ Continuous fetal monitoring (instead of intermittent)
- ☐ Epidural when I planned without
- ☐ Forceps or vacuum assistance
- ☐ Episiotomy
- ☐ C-section when I planned a vaginal birth
- ☐ Separation from the baby after birth
- ☐ Routine procedures I have not consented to
- ☐ Other: \_\_\_\_\_

## WHAT I HAVE WRITTEN IN MY BIRTH PLAN ABOUT THESE

See P14. Bring the plan.

My specific preferences on the interventions above:

---

---

---

## WHAT I HAVE TALKED TO MY PROVIDER ABOUT REGARDING THESE

---

---

WHAT THEIR HOSPITAL'S RATES LOOK LIKE (for interventions you are worried about)

You can ask. "What are the c-section rates at this hospital?  
What are the episiotomy rates? What are the induction rates?"

Numbers they shared:

---

*Fear of interventions you might not want,  
page 2*

---

## THE HARD TRUTH

Sometimes interventions you did not want are the right call. The team is making a decision based on what they see in real time. An emergency c-section that saves your baby is not the same as a "convenience c-section." A repair of significant tearing is the right move. Pitocin to manage stalled labor sometimes prevents a worse intervention later.

The fear is not about the interventions themselves. It is about having interventions PUSHED on you that were not necessary, or about not understanding what is happening to your body in real time.

## WHAT TO ASK IN THE MOMENT

"Is this an emergency or do we have time to talk?"

"What happens if we wait 30 minutes / 1 hour?"

"What are you seeing that is making you suggest this?"

"Can we try [alternative] first?"

## WHAT TO ASK YOUR PARTNER OR DOULA TO ASK IF YOU CANNOT

---

## WHAT TO AGREE ON IN ADVANCE, AS A COUPLE

The interventions we will accept without question in an emergency:

---

---

The interventions we want to talk about before agreeing to:

---

---

The "code phrase" between us for "I need you to slow this down with the team":

---

A NOTE: AFTER THE BIRTH, IF YOU FELT PUSHED INTO INTERVENTIONS THAT YOU DID NOT WANT, THAT IS A REAL FEELING AND CAN BE PROCESSED. BIRTH

TRAUMA IS REAL. PERINATAL THERAPISTS HELP WITH IT. P09 POSTPARTUM ANXIETY + PPD JOURNAL HAS PROCESSING PROMPTS.

## *Fear of the unknown, page 1*

---

### WHAT THE UNKNOWN FEELS LIKE

- ☐ I have never given birth before.
- ☐ I have given birth and it was traumatic.
- ☐ I have given birth and it was different from what I expected.
- ☐ I have read so much that the unknown feels both bigger and stranger.
- ☐ I cannot picture how it will go.

### WHAT I CAN PRE-VIEW

- ☐ A hospital tour (most hospitals offer them)
- ☐ A childbirth education class
- ☐ A doula consultation
- ☐ A conversation with a friend who has given birth
- ☐ Reading a couple of birth stories from people whose values match yours (avoid the trauma-spiral side of the internet)

### WHAT I CANNOT PRE-VIEW

- The exact day labor starts
- How long it will be
- What it will feel like for me
- What my body will do
- Who will be the on-call provider

These are unknowable in advance. The work is making peace with the unknowable, not erasing it.

### ONE THING I WILL DO TO REDUCE THE UNKNOWN

---



## *Fear of the unknown, page 2*

---

## TOOLS FOR LIVING WITH THE UNKNOWN

### PREPARE WHAT YOU CAN

- The bag is packed.
- The hospital is known. The route is known. The backup route is known. The parking situation is known.
- The team is told where you are giving birth.
- The pediatrician is chosen.
- The first 48 hours after the baby comes home has a plan.

### LET GO OF WHAT YOU CANNOT

- The exact day. Babies arrive when they arrive.
- The duration. Some labors are 4 hours, some are 36.
- The pain level. You will not know until you are in it.
- Who is on call. Most hospitals have a rotation. You may meet them when you meet them.
- The exact path. The plan will adjust.

### BUILD A "WHATEVER HAPPENS" MANTRA

The version of this for me is:

---

---

### A WORD ABOUT BIRTH STORIES

Reading birth stories can be helpful when the stories you read are from people whose values and outcomes match yours, told with honesty without dramatizing.

Reading birth stories can be harmful when they are skewed toward the dramatic, the traumatic, or the "I knew something was wrong" genre.

For this stretch of pregnancy, consider a limit on how much birth content you take in. The "I read every story I could find" approach is rarely the calming one.

### WHAT I AM CONSUMING + WHAT I AM CUTTING BACK ON

---

---

# What would help me, specifically

---

A bridge from fear to action.

## WHAT WOULD ACTUALLY HELP ME

- ☐ A doula (or a specific doula I am considering)
- ☐ Childbirth education classes (or a specific one)
- ☐ A perinatal therapist
- ☐ A conversation with my provider where I name all the fears
- ☐ A pre-labor hospital tour
- ☐ A specific person being there or not being there
- ☐ A different provider, frankly
- ☐ A different hospital, frankly
- ☐ A real conversation with my partner
- ☐ Time to grieve a prior birth before this one
- ☐ Permission to feel scared without being told to relax

## THE THREE THINGS I WILL DO IN THE NEXT TWO WEEKS

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_

A REMINDER: IF YOUR PROVIDER OR HOSPITAL ROUTINELY DISMISSES YOUR FEARS, THAT IS INFORMATION. SECOND OPINIONS EXIST. CHANGING PROVIDERS LATE IN PREGNANCY IS LESS FUN THAN EARLIER BUT IS SOMETIMES THE RIGHT MOVE. YOUR GUT KNOWS WHEN SOMETHING IS OFF.

## *Scripts for telling the team you are scared*

---

**P**eople often do not tell their providers how scared they are. This page is the words.

#### SCRIPT 1, AT AN APPOINTMENT

"I have been more anxious about the birth than I expected. I want to tell you some of the specific fears I have, so we can talk about each one before labor."

#### SCRIPT 2, BEFORE A SPECIFIC PROCEDURE OR DISCUSSION

"I am nervous about this. Can you walk me through it slowly and tell me what is normal vs what I should worry about."

#### SCRIPT 3, ASKING FOR A REFERRAL

"I think my anxiety is bigger than what we can address in appointments. Can you refer me to a perinatal therapist?"

#### SCRIPT 4, WHEN A NURSE OR DOCTOR DISMISSES YOUR FEAR

"I hear you that statistically it's unlikely. I am asking you to take this concern seriously anyway."

#### SCRIPT 5, ASKING TO BE BELIEVED

"I have a history of [trauma / prior loss / dismissal in medical settings]. I am telling you so you can take that into account in how you talk with me."

#### SCRIPT 6, AT THE HOSPITAL ARRIVAL

"I am scared. I want the team to know that. I am asking you to be patient with me during this labor."

#### SCRIPT 7, FOR YOUR PARTNER TO USE ON YOUR BEHALF

"She is afraid of [specific thing]. I am asking you to take that seriously when you talk with her about [decision]."

A NOTE: TELLING THE TEAM YOU ARE SCARED IS NOT WEAKNESS. IT IS INFORMED CONSENT. PROVIDERS WHO LISTEN TO FEAR DELIVER BETTER CARE. PROVIDERS WHO DISMISS IT ARE A DIFFERENT QUESTION YOU CAN ANSWER BEFORE LABOR.

## *The night-before-labor page*

---

**F**or the version of you on the eve of induction, or the version of you in early labor at home.

TONIGHT'S DATE: \_\_\_\_\_

WHO IS WITH ME

Partner: \_\_\_\_\_

Other support person: \_\_\_\_\_

WHAT I NEED FROM THEM, TONIGHT, SPECIFICALLY

\_\_\_\_\_  
\_\_\_\_\_

WHAT I HAVE PACKED

- ☐ Hospital bag
- ☐ The birth plan (printed)
- ☐ This workbook (in the bag)
- ☐ Phone fully charged
- ☐ Snacks for partner
- ☐ Going-home clothes for me and baby
- ☐ Car seat installed
- ☐ A handwritten note to myself (see below)

A NOTE FROM ME, NOW, TO ME, IN LABOR

When you read this, you are scared. That is fine. The fear is information, not prediction. You have prepared. You have a team. You have me on your side, from before this. You are not alone.

Specifically, I want me-in-labor to remember:

\_\_\_\_\_  
\_\_\_\_\_  
\_\_\_\_\_  
\_\_\_\_\_

THE LAST THING I WANT TO DO BEFORE I GO TO BED

\_\_\_\_\_

A REMINDER FOR TONIGHT



Eat something even if you are not hungry. Drink water. Take a warm shower. Lay out clothes. Charge the phone. Set the alarm. Hug your partner.

This is the last night of being just you. Tomorrow you will have done something enormous. Tonight, sleep.

## *The “I am not okay” page*

---

**B**irth anxiety has a range. At one end, the worry is uncomfortable but manageable. At the other end, it is consuming. The workbook helps the middle. The far end is for a perinatal mental-health professional.

## THE HONEST CHECK

- Is the anxiety interfering with sleeping? ☐ Yes ☐ No
- Is it interfering with eating? ☐ Yes ☐ No
- Is it making me avoid prenatal care? ☐ Yes ☐ No
- Am I having panic attacks? ☐ Yes ☐ No
- Am I having intrusive thoughts about harm? ☐ Yes ☐ No
- Am I unable to think about anything else? ☐ Yes ☐ No
- Have I had nightmares about the birth? ☐ Yes ☐ No
- Am I considering avoiding the birth somehow? ☐ Yes ☐ No

If two or more of these are true, the next step is a perinatal mental-health professional. Not the workbook. Not your provider alone. A specialist.

## PERINATAL ANXIETY + DEPRESSION ARE TREATABLE

The treatment is good. The sooner you get to it, the faster the anxiety becomes manageable. Some treatment is talk-only. Some includes medication that is okayed by your OB and your prescriber together. Both are valid.

## CRISIS LINES

US, 988 Suicide and Crisis Lifeline (call or text)

US, Postpartum Support International HelpLine  
1-833-852-6262 (call or text)  
(PSI also serves prenatal anxiety, not just postpartum)

US, Maternal Mental Health Hotline: 1-833-943-5746

UK, Samaritans: 116 123

Canada, 9-8-8 Suicide Crisis Helpline

Australia, PANDA: 1300 726 306

## A FINDING-A-THERAPIST CHECKLIST

- ☐ Ask your OB for a referral

- ☐ Search Postpartum Support International's provider directory
- ☐ Check your insurance's mental-health network
- ☐ Look for "perinatal" or "reproductive mental health" specialty
- ☐ A consultation call before committing is reasonable

THE THERAPIST I AM RESERVING THE NUMBER FOR

---

---

A FINAL REMINDER: SEEING A THERAPIST IS NOT FAILURE. SOME OF THE CALMEST PEOPLE IN LABOR ARE THE ONES WHO HAVE BEEN IN PERINATAL THERAPY FOR MONTHS BEFOREHAND. THE PREPARATION IS THE GIFT.

## *A final letter*

---

**Y**ou did the work of naming what you are afraid of.

Most pregnant people who are afraid of birth do not name it that specifically. They carry it around as a fog. You sat down with this workbook and made it specific. That is the harder thing. The fog cannot be addressed. The specific fear can.

Whatever happens during the birth, you are not the version of you who picked up this workbook and refused to do anything about the fear. You did something about it. You named it. You brought it to the people who could help. You wrote scripts. You made a plan.

You may still be afraid in labor. That is allowed. You will also be capable. Both can be true.

The labor will end. The baby will arrive. The fear, for the most part, lifts in the days after, replaced by other things. Some of the fear is the thing you carried. Some of it was preparation. Both are forms of love.

You are not alone in this, even when the room is full of strangers.

Soothemade



# Soothemade *Notes*

*Made for the version of you no one tells you about, the  
one at four in the morning, the one in the parking lot, the  
one who doesn't recognize her own week.*

[notes.soothemade.com](https://notes.soothemade.com)

---

Personal use only. Not for redistribution.  
© 2026 Soothemade · made slowly, in plain language